St. Jude Academic Health Center - Division of Hospital Medicine

CLINICAL DOCUMENTATION — NARRATIVE SURGICAL DISCHARGE SUMMARY V1

Patient: Joseph Taylor | **MRN:** PAT-60015 | **DOB:** 1964-12-07 | **Gender:** Male | **Doc ID:** NARRATIVE_0015
Encounter Date: 2025-05-24 | **Discharge/Note Date:** 2025-05-29 | **Provider:** Dr. Lisa Cuddy, MD

PREOPERATIVE DIAGNOSIS & PROCEDURE

The patient underwent uncomplicated arthroscopic knee meniscectomy and chondroplasty under general endotracheal anesthesia.

HISTORY OF PRESENT ILLNESS

The patient is an adult who presented electively for surgical intervention following conservative management failure. Pre-admission testing confirmed that preoperative total cholesterol was 235.6 mg/dL (HIGH). Preoperative metabolic screening showed a baseline platelet count of 521 10*3/uL (HIGH). The patient was cleared from a cardiopulmonary standpoint and taken to the operative suite.

POSTOPERATIVE HOSPITAL COURSE

The patient was extubated in the operating room and transferred to the post-anesthesia care unit in stable condition. Postoperative recovery was uneventful. For pain and infection prophylaxis, the patient was prescribed Amlodipine 5 mg orally once daily. On postoperative day one, postoperative creatinine remained stable at 0.92 mg/dL. Surgical incisions were clean, intact, and without erythema or purulent drainage.

POST-DISCHARGE MEDICATIONS

Discharge medications: The patient will take Amoxicillin 250 mg by mouth three times daily for 7 days, and continue baseline Omeprazole 40 mg orally once daily in the morning for 30 days. All prescriptions were transmitted electronically to the patient's pharmacy.

ACTIVITY & WOUND CARE PLAN

Keep dressing clean and dry. No heavy lifting greater than 10 pounds for two weeks. Surgical clinic follow-up scheduled in 10 days.